



Care Indeed Home Health

Mock Patient Evaluation Document · Version 1.0 MOCK · Training Date
July 1, 2026

MOCK TRAINING RECORD
SYNTHETIC UAT DATA
NOT FOR CLINICAL USE
DO NOT UPLOAD TO LIVE PATIENT
CHART

EVALUATION 3 — VERY HIGH ACUITY

Primary Diagnosis: New-Onset Stroke (Left MCA CVA) with Right Hemiparesis, Dysphagia, and Aphasia

PATIENT DEMOGRAPHICS	
Field	Detail
Patient Name	Yuki Tanaka-Morrison
Date of Birth	08/09/1948
Age	76
Sex	Female
Race/Ethnicity	Asian / Japanese American
Primary Language	English (fluent); Japanese (native — reverts to Japanese when fatigued or stressed per family)
Interpreter Needed	Yes — Japanese interpreter recommended for complex medical discussions and when patient is fatigued
Marital Status	Married
Medical Record Number	CI-2025-00431
Address of Service	560 Cherry Blossom Lane, Foster City, CA 94404
Home Phone	(650) 555-0388
Cell Phone	N/A (patient unable to use; husband cell below)
Email	N/A
County	San Mateo

EMERGENCY CONTACTS

Priority	Name	Relationship	Phone	Lives with Patient?
1	David Morrison	Husband	(650) 555-0389 (home) / (650) 555-0156 (cell)	Yes
2	Keiko Tanaka-Morrison	Daughter	(415) 555-0733	No — SF (visiting daily through 07/15)
3	Dr. Thomas Morrison	Son (physician — internal medicine, lives out of state)	(312) 555-0921	No — Chicago, IL

LIVING SITUATION

Field	Detail
Lives With	Husband David (age 79 — mild arthritis, uses cane, hearing aids; active and cognitively intact; devoted but physically limited)
Residence Type	Two-story home, owned (bedroom upstairs; temporary bedroom setup in ground-floor study)
Stairs to Enter	None (ground-level entry at garage)
Interior Stairs	14 stairs to second floor (not currently being used — patient confined to ground floor)
Pets	None
DME in Home	Hospital bed (ground-floor study), Hoyer lift (rented), wheelchair (standard — needs assessment for tilt-in-space), bedside commode, suction machine (portable), pulse oximeter, transfer belt

PAYER INFORMATION

Priority	Payer	ID / Policy Number	Group Number
Primary	Medicare Part A (Traditional)	7HK2-AB9-QW15	N/A
Secondary	AARP/UnitedHealthcare Medicare Supplement (Plan G)	UHC-223456781	N/A

PHYSICIAN INFORMATION

Role	Name	Specialty	Phone	Fax
Attending / Certifying	Dr. Sarah Lin	Neurology (Stroke)	(650) 555-1310	(650) 555-1311
PCP	Dr. Robert Huang	Internal Medicine	(650) 555-0580	(650) 555-0581
Physiatrist	Dr. Karen Okonkwo	PM&R	(650) 555-1442	(650) 555-1443

REFERRAL / HOSPITALIZATION HISTORY

Event	Facility	Dates	Reason
Acute Hospitalization	Stanford Medical Center	05/28/2025 – 06/03/2025	Left MCA ischemic stroke; tPA administered; ICU x 3 days
Inpatient Rehabilitation	Peninsula Rehabilitation Center	06/03/2025 – 06/16/2025	Intensive stroke rehabilitation (13 days)

Start of Care Date: 06/17/2025 Referral Source: Peninsula Rehabilitation Center — Dr. Okonkwo / Discharge Planner: Angela Rodriguez, MSW

DIAGNOSES

Priority	ICD-10 Code	Diagnosis
Primary	I63.512	Cerebral infarction due to unspecified occlusion or stenosis of left middle cerebral artery
Secondary	I69.351	Hemiplegia and hemiparesis following cerebral infarction affecting right dominant side
Secondary	I69.321	Dysphasia following cerebral infarction (expressive aphasia, moderate)
Secondary	I69.391	Dysphagia following cerebral infarction
Secondary	R13.19	Other dysphagia (modified diet: mechanical soft with thin liquids per SLP assessment at rehab discharge)
Secondary	I48.0	Paroxysmal atrial fibrillation (new diagnosis — etiology of stroke; detected inpatient)
Secondary	I10	Essential hypertension
Secondary	E78.00	Pure hypercholesterolemia, unspecified
Secondary	M79.3	Panniculitis, unspecified (chronic right shoulder pain — premorbid)
Secondary	N39.0	Urinary tract infection, site not specified (treated inpatient, resolved; Foley discontinued 06/10/2025)
Secondary	R39.15	Urgency of urination (new since stroke — neurogenic bladder suspected)
Secondary	F06.30	Mood disorder due to known physiological condition, unspecified (post-stroke depression/emotional lability)

Surgical History:

Cesarean section x2 (1978, 1981)
Laparoscopic cholecystectomy (2009)
Right carpal tunnel release (2015)

ALLERGIES

Allergen	Reaction	Severity
Morphine	Respiratory depression, severe sedation	SEVERE
Metoclopramide (Reglan)	Dystonic reaction (tongue protrusion, neck stiffness)	SEVERE
Peanuts	Anaphylaxis	SEVERE
Latex	Urticaria	Moderate

CURRENT MEDICATIONS

#	Medication	Dose	Route	Frequency	Purpose	High-Risk?
1	Apixaban (Eliquis)	5 mg	PO (crush and mix with applesauce per SLP)	BID	Anticoagulation (A-fib — stroke prevention)	YES
2	Atorvastatin (Lipitor)	80 mg	PO (crush)	Daily HS	Hyperlipidemia / secondary stroke prevention	No
3	Amlodipine	10 mg	PO (crush)	Daily AM	HTN	No
4	Lisinopril	20 mg	PO (crush)	Daily AM	HTN	No
5	Metoprolol succinate (Toprol XL)	50 mg	PO (do NOT crush — extended release)	Daily AM	Heart rate control (A-fib)	Yes
6	Sertraline (Zoloft)	50 mg	PO (crush)	Daily AM	Post-stroke depression	No
7	Baclofen	10 mg	PO (crush)	TID	Spasticity (right upper extremity)	No
8	Oxybutynin (Ditropan)	5 mg	PO	BID	Urinary urgency / neurogenic bladder	No
9	Bisacodyl (Dulcolax)	10 mg	PO/PR	PRN (if no BM x 3 days)	Constipation	No
10	Acetaminophen	650 mg	PO (crush in applesauce)	Q6H PRN	Pain (shoulder, general)	No
11	Multivitamin	1 tab	PO (crush)	Daily	Nutritional support	No

Critical Medication Note: Most PO medications must be crushed and administered with applesauce or pudding due to dysphagia. Exception: Metoprolol succinate (Toprol XL) is EXTENDED RELEASE — must be swallowed whole. If patient cannot swallow whole, notify physician for alternative formulation (metoprolol tartrate — immediate release, crushable, but requires BID-TID dosing).

Controlled Substances in Home: None Sharps in Home: None

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter	Value	Abnormal?
BP	148/82 mmHg (left arm — right arm not used for BP per stroke protocol)	Mildly elevated
HR	72 bpm, irregular (intermittent)	Irregular — known paroxysmal A-fib
RR	16	WNL
Temp	98.4°F	WNL
SpO2	97% on RA	WNL
Weight	138 lbs (62.6 kg) — down from pre-stroke 152 lbs	Weight loss 14 lbs in 3 weeks (9.2%) — significant
Height	5'3" (160 cm)	
BMI	24.4	WNL
Pain	5/10 right shoulder (spasticity-related); 2/10 generalized	Yes

CLINICAL ASSESSMENT — NEUROLOGICAL / STROKE-SPECIFIC

Motor:

Area	Finding
Right Upper Extremity	Severe hemiparesis. Shoulder: 2/5 (gravity-eliminated movement only). Elbow: 2/5. Wrist: 1/5 (trace). Hand/grip: 1/5 (trace — unable to grasp objects). Fine motor: absent. Increased tone/spasticity (Modified Ashworth Scale 2 — marked increase through ROM, limb easily moved). Subluxation: right shoulder 1 fingerbreadth inferior subluxation. Sling in use during upright activities.
Right Lower Extremity	Moderate hemiparesis. Hip: 3/5. Knee: 3-/5. Ankle: 2+/5 (dorsiflexion weak — foot drop present). Tone: mildly increased (MAS 1+). AFO (ankle-foot orthosis) prescribed, not yet received.
Left Upper Extremity	5/5 — fully intact
Left Lower Extremity	5/5 — fully intact

Sensation:

Right side: diminished light touch (upper and lower extremity), diminished proprioception (right hand and foot), intact pain/temperature.
Left side: intact all modalities.

Speech / Language (Aphasia Assessment):

Type: Expressive (Broca's) aphasia, moderate severity.

Comprehension: Relatively preserved — follows 2-step commands consistently, occasional difficulty with 3-step commands.

Expression: Limited to 2-3 word phrases; frequent word-finding difficulty; telegraphic speech ("want... water... please"); high frustration tolerance noted. Can nod/shake head for yes/no reliably.

Reading: Able to read single words and short phrases; difficulty with paragraphs.

Writing: Unable with dominant (right) hand due to hemiparesis. Left-hand writing: illegible — just beginning to practice.

Communication aids currently in use: Yes/No board (laminated); picture communication board (basic ADL needs, pain, emotions); husband/daughter assist with interpretation of gestures.

Pre-stroke language: Fully bilingual English/Japanese. Currently reverts to Japanese words/phrases when fatigued, frustrated, or emotional. Family confirms she understood both languages equally pre-stroke.

Swallowing / Dysphagia:

Modified Barium Swallow (MBS) at rehab (06/12/2025): Mild-moderate oropharyngeal dysphagia with delayed pharyngeal swallow trigger; reduced base of tongue retraction; trace penetration with thin liquids (PAS 3), no aspiration observed.

Current Diet Order: Mechanical soft solids / thin liquids (IDDSI Level 5 minced & moist; IDDSI Level 0 thin liquids). (Note: updated from initial thickened liquids at acute admission — thin liquids cleared at rehab discharge per SLP.)

Swallowing Precautions: Upright $\geq 90^\circ$ during meals and 30 minutes after; small bites/sips; alternate solids and liquids; check for oral pocketing (right cheek) after each bite; no straws (per SLP recommendation — sip from cup); cue for chin tuck if coughing.

Aspiration Risk: Moderate. Signs to monitor: wet/gurgling voice after swallowing, coughing during/after meals, throat clearing, pocketing food in right cheek, refusal to eat, recurrent fevers (aspiration pneumonia risk).

Suction machine: Available at bedside (portable) — for emergency use only (oral suctioning); husband trained on use at rehab.

Cognition:

Alert and oriented x3 (person, place, time) — occasional confusion about date.

Attention: mildly impaired (difficulty sustaining attention for >10-15 minutes).

Memory: short-term recall impaired (2/3 objects at 5 minutes); long-term intact.

Judgment: mildly impaired — patient occasionally attempts to stand unassisted (impulsive, safety concern).

Executive function: impaired — difficulty with multi-step problem solving and sequencing.

Visual-spatial: right-sided neglect suspected — frequently fails to notice objects on right side of visual field; bumps right arm/leg into doorframes and furniture. Formal testing recommended.

Emotional / Behavioral:

Emotional lability: present. Patient cries easily (triggered by frustration with speech, discussion of stroke, or seeing family members). Episodes are brief (1-2 minutes), not sustained. Patient indicates awareness of lability ("sorry... can't stop... crying").

Post-stroke depression: PHQ-2 positive (unable to complete full PHQ-9 due to aphasia — approximated score via clinician-administered modified version: ~12, moderate). Sertraline started 2 weeks ago.

Frustration: significant — primarily related to aphasia and functional dependence.

Motivation: good — actively engages in therapy, demonstrates effort.

No agitation, no aggression, no psychosis.

HOMEBOUND STATUS ASSESSMENT

Criterion	Finding
Criterion 1	MET. Right hemiparesis with severe upper extremity involvement and moderate lower extremity involvement. Requires Hoyer lift for bed-to-wheelchair transfers. Unable to ambulate independently (requires rolling walker + max assist of 1 for standing pivot transfers only — not yet ambulatory). Right foot drop without AFO. Aphasia limits ability to communicate needs and navigate in community. Considerable and taxing effort to leave home — requires wheelchair, Hoyer lift or 2-person assist for vehicle transfer, and a companion for communication.
Criterion 2	MET. Absences limited to medical appointments only (neurology, physiatry).
Homebound Status	CONFIRMED

ADVANCE DIRECTIVES

Document	Status
AHCD	Yes — executed 2019. Husband David designated as primary agent; daughter Keiko as alternate.
POLST	Yes — completed 06/02/2025 (during acute hospitalization). Full treatment (Section A); full treatment including intubation (Section B); long-term artificial nutrition: trial period, reassess in 90 days (Section D).
DNR	No — Full Code

SERVICES ORDERED

Service	Discipline	Frequency	Duration
Skilled Nursing	RN	3x/week x 4 weeks, then 2x/week x 4 weeks	8 weeks
Physical Therapy	PT	5x/week x 4 weeks, then 3x/week x 4 weeks	8 weeks
Occupational Therapy	OT	5x/week x 4 weeks, then 3x/week x 4 weeks	8 weeks
Speech-Language Pathology	SLP	5x/week x 4 weeks, then 3x/week x 4 weeks	8 weeks
Medical Social Work	MSW	1x/week x 4 weeks, then 2x/month	8 weeks
Home Health Aide	HHA	Daily (AM — personal care, mobility assist)	Duration of need

SKILLED NURSING FOCUS AREAS

Neurological Monitoring — Neuro checks each visit (LOC, orientation, pupil response, motor strength bilateral, speech quality, new deficits). NIH Stroke Scale (NIHSS) trend monitoring. Recognize signs of recurrent stroke.

Anticoagulation Management — Apixaban compliance; monitor for bleeding (critical given new A-fib anticoagulation); educate husband on signs of major bleeding, when to call 911.

Dysphagia Management — Assess swallowing safety each visit; monitor weight weekly (significant weight loss trajectory); calorie count x 3 days if intake remains poor; coordinate with SLP for diet advancement or regression; aspiration pneumonia surveillance (lung sounds, temperature, SpO2).

Medication Crushing Protocol — Verify which medications can be crushed (NOT Toprol XL); teach husband correct technique; assess for alternative formulations with physician for medications that cannot be crushed.

Skin Integrity — Braden Scale: 13 (moderate risk). Assess right shoulder sling fit (skin breakdown risk).

Repositioning schedule q2h when in bed, q1h when in wheelchair. Pressure-relieving cushion for wheelchair. Inspect bony prominences each visit.

Urinary Management — Monitor neurogenic bladder symptoms; timed voiding schedule; I&O monitoring if ordered; UTI surveillance (previous UTI inpatient).

Depression/Emotional Lability Monitoring — Assess response to sertraline; coordinate with MSW/SLP for communication-based coping strategies.

Caregiver Training — Husband David is 79 with arthritis and uses a cane. Caregiver burden is a critical concern.

Teach: Hoyer lift operation (both Keiko and David), transfer techniques, dysphagia precautions/meal preparation, medication crushing, suctioning, skin inspection, when to call agency vs. 911. Assess David's physical and emotional capacity at each visit.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal	Target
1	No recurrent stroke or TIA events	08/17/2025
2	No aspiration pneumonia events	08/17/2025
3	Weight stabilized (no further loss; target ≥ 140 lbs)	08/01/2025
4	Right UE strength improved to $\geq 3/5$ at shoulder and elbow	08/17/2025
5	Right LE strength improved to $\geq 4/5$ at hip and knee	08/17/2025
6	Ambulate 50 feet with AFO, rolling walker, and contact guard assist	08/17/2025
7	Standing pivot transfer with moderate assist of 1 (eliminate Hoyer lift)	08/01/2025
8	Produce 4-5 word sentences with 60% intelligibility on functional topics	08/17/2025
9	Diet advanced to regular/modified texture per SLP reassessment	08/17/2025
10	Right neglect compensatory strategies demonstrated independently	08/17/2025
11	ADL participation: upper body dressing with setup only (left-hand dominant retraining)	08/17/2025
12	Husband demonstrates all caregiver tasks independently and safely	07/15/2025
13	PHQ-equivalent depression score improved to < 10	08/17/2025
14	Skin intact, no pressure injuries	08/17/2025
15	Zero falls	08/17/2025
16	Effective communication system established (AAC device or advanced communication board)	07/31/2025

PATIENT SIGNATURES — EVALUATION 3

Patient Signature (left hand): _____ Date: _____

(Patient signed with left hand — non-dominant; verified patient identity and understanding verbally and via communication board)

Print Name: Yuki Tanaka-Morrison

Husband / Health Care Agent Signature: _____ Date: _____

Print Name: David Morrison

Interpreter Used: ☐ Yes — Japanese telephone interpreter (Language Line) for complex medical discussion portions.

Clinician Signature: _____ Date: _____

Patient / Representative / Conservator

Mock signature area — training only.

Evaluating Clinician

Mock signature area — training only.

Caregiver / Interpreter if applicable

Mock signature area — training only.

Date / Attestation

Mock signature area — training only.